

Athens, 09-04-2025

No. Protocol: 1243

DECISION 11/2025

(Department)

The Hellenic Data Protection Authority convened, at the invitation of its President, in a regular session in the composition of a Department via teleconference, at its headquarters on 15-01-2025, in order to examine the case referred to in the background of this document. Present at the meeting were Georgios Batzalexis, Deputy President, due to the absence of the President of the Authority Konstantinos Menoudakis, and Demosthenes Bougioukas and Maria Psalla, alternate members of the Authority, replacing Konstantinos Lamprinoudakis and Grigorios Tsolias, respectively, who, although duly summoned in writing, did not attend due to impediment. Also present at the meeting was Nikolaos Libos, as rapporteur. Present, without voting rights, were Eleni Kapralou, legal expert, as assistant rapporteur, and Irini Papageorgopoulou, employee of the Administrative Affairs Department, as secretary.

The Authority took into account the following:

With the complaint No. G/EIS/7713/05-06-2022 regarding a violation of the provisions under the Authority's jurisdiction, A (hereinafter "complainant") complains against B (hereinafter "complained party"), who is a private physician specializing in gynecology and monitored the complainant's pregnancy during the period from September 2020 to October 2021, complaining about: a) incomplete satisfaction of the right of access, which consists of the non-provision of her complete medical file, b) unlawful access to her personal electronic health file through the website <https://ehealth.gov.gr>, as well as c) illegal transmission to third parties of the personal data concerning the complainant, related to her health status.

Specifically, the complainant claims that on October 13, 2021, after the birth of a stillborn fetus (...), she informed the aforementioned gynecologist in writing that she did not wish to continue their collaboration, simultaneously requesting a copy of her complete medical file that the said physician was obliged to maintain in her records, according to Article 14 of Law 3418/2005 (Code of Medical Ethics). However, the said physician provided, according to the complaint, on 21-10-2021 to the complainant's husband an incomplete medical file, as it did not include all the tests that had been conducted during the monitoring of her pregnancy, and specifically the complained party did not provide the ultrasounds that had been performed, claiming that she does not keep a record of the ultrasounds due to sensitive personal data and that she has already provided them to the complainant.

Subsequently, the complainant states that on June 1, 2022, she discovered that during the period from October 2021 to May 2022, the aforementioned physician, although she was no longer the complainant's treating physician (their collaboration had already ended on 13-10-2021), exploited her access rights to the electronic medical file system (<https://ehealth.gov.gr>) which is provided to her by virtue of her position, and repeatedly gained unlawful access to the complainant's personal data without her consent. The complainant also claims that according to Article 84 of Law 4600/2019 (Government Gazette A', 43) regarding the Individual Electronic Health File (IEHF), Law 3418/2005 (Code of Medical Ethics, Government Gazette A', 287), and Law 3892/2010 regarding the electronic recording and execution of medical prescriptions and referrals for medical examinations (Government Gazette A', 189), the access of any treating physician to the personal data of their patient, for the purpose of providing health services, is not based on the patient's consent for the processing of their data, but on the independent legal basis of providing health services (Article 9 par. 2 letter (h) of the GDPR) by the respective treating physician, and that any other physician who gains access to personal data...

character of a person, who does not have the status of the patient, without the consent of that person, constitutes unauthorized access. In contrast to the above, according to the allegations, the

aforementioned doctor during the period from October 5, 2021, to May 31, 2022, although she did not have the status of the treating physician and had not obtained the consent of the complainant for the specific access to the health data concerning her, repeatedly gained unauthorized access to the electronic medical file of the complainant in the information system <https://ehealth.gov.gr>, extracting medical information related to the health status of the latter and subsequently transmitted this information to third parties.

The Authority, in the context of examining the above complaint, sent to the respondent the document numbered Γ/ΕΞΕ/2143/30-08-2022 for the provision of opinions. Subsequently, the respondent replied to the above document with her document numbered Γ/ΕΙΣ/10473/27-09-2022, specifically:

a) Regarding the complaint about the incomplete satisfaction of the complainant's right of access, the respondent claimed that:

1. She maintained a complete medical record, with all legally required elements, including all clinical and paraclinical examinations that were conducted.
2. All examinations conducted both during the complainant's pregnancy (a total of nine visits) and during her only visit after the unfortunate incident with the fetus, as well as the results thereof, such as ultrasounds, cardiotocograms, fetal growth curves, pregnancy reports, etc., were provided to the complainant in view of her visit—following her own referral—to a specialist hematologist for further investigation of the causes of the termination of the pregnancy with the loss of the fetus. In fact, as she claims, at each pregnancy ultrasound, the complainant was given a report on fetal development and a photograph, along with examinations every month, while at the penultimate visit, a cardiotocogram was performed and given to the patient at that moment.

4

3. Around mid-October 2021, the complainant's husband informed the respondent that for psychological reasons it would be better for the complainant to visit another gynecologist for her annual follow-up, even requesting that she not contact the complainant so as not to disturb her.
4. Subsequently, around late October to early November 2021, based on the consent from the complainant that the respondent assumed existed due to the continuous accompaniment by her husband during visits to her office, she handed over to the complainant's husband all the remaining examinations that she had in her records, as all the others were already in the hands of the complainant, either in view of the investigation of the causes of the sudden termination of the pregnancy or upon her departure from the office after her visits.
5. The complainant claims that the results of the ultrasounds were not delivered to her husband and maliciously does not mention that she already had them in her records.
6. The respondent finally claims that she is prevented from detailing all the medical checks and examinations conducted due to the obligation to maintain medical confidentiality in the context of a potential administrative audit against her.

b) Regarding the complainant's complaint about unauthorized repeated access to her individual electronic health file in the system <https://ehealth.gov.gr>, the respondent claimed that:

1. Access to the said electronic file pertains to two time periods. The first, during the period from October to December 2021, concerns her attempt to gather all the material and history of the complainant, so that either in view of the delivery of the file to her husband or in view of informing the next treating physician, she would be able to provide accurate information about the actual facts and the therapeutic protocol followed. Subsequently, the second time period during which she had access was after about 5-6 months, around late May to early June 2022, when the respondent learned completely by chance from another patient of hers.

5

that she last encountered the complainant coincidentally at an ultrasound clinic, and in this way, the respondent learned that the complainant might be pregnant again. In light of this accidental information (which occurred during this second time period), as claimed by the respondent, her obligation to assist the complainant in any way requested came back to her mind, and given her erroneous, as evidenced, perception of the situation regarding the presumed willingness of the complainant to stand by her,

even though she was no longer her treating physician, she accessed her electronic file two or three times in total during the aforementioned second time period at the end of May and early June 2022, in order to recall what had happened in light of the possible new pregnancy.

2. After the loss of the fetus, the complainant requested her assistance in recommending a gynecologist, indicative of the prevailing atmosphere and the image created for her role even after the transfer of the file, but also on a social level. This, she claims, was the main reason she continued to try - even after the end of their collaboration - to scientifically assist in uncovering the causes of the neonatal loss, as she was well aware of how much the complainant desired to become a mother and how much this unexpected event cost her. In light of these circumstances, she considered it her duty, as she explains, to provide any assistance she could, even informally to another colleague treating physician, for a case that shook her, mainly because the cause of the neonatal loss was never definitively established.

3. She accessed the complainant's medical file a total of 6-7 times and not as many times as indicated in the documents submitted by the complainant.

4. She had no expected gain or benefit from her above actions; on the contrary, she was genuinely happy about the possibility of a new pregnancy, being responsibly thoughtful regarding her role as the former treating physician who handled the first pregnancy, and the tragic event of the loss of the fetus marked her as a physician and a person, and all she thought about was how she could be helpful in the complainant's efforts.

6

g) Finally, regarding the complaint about the transmission of her personal data to third parties (health data), the respondent denies any transmission of the complainant's data to third parties. She claims that she simply asked the complainant's godmother and patient, who already knew about the complainant's pregnancy status, whether the complainant was pregnant, within the context of the distorted image she had regarding the good intentions of the complainant towards her. Subsequently, the Authority sent the complainant the document numbered G/EXE/2721/27-10-2022 providing her with the opportunity to submit written opinions on the aforementioned response of the respondent. The complainant responded to this document from the Authority with the document numbered G/EIS/12057/24-11-2022 that she sent, reiterating the claims made in the complaint, additionally stating the following, specifically that:

1. The obligation of the respondent cannot be negated by the fact that she already had certain elements in her possession, as in any case she was obliged to provide her with a copy of all the medical acts she had performed in her office (ultrasounds, Doppler tests, cardiotocograms, fetal growth curves, etc.).

2. She also states that during her visits to the respondent's office for monitoring the pregnancy, she was given copies of some measurements as well as some photographs of the ultrasounds. From these documents, some do not bear her name, while in others it has been added by hand. The complainant even submits the documents from the medical file that were handed over to her by the respondent, specifically submitting the ultrasound from 02-02-2021 (measurements, charts, and images), the ultrasound from 16-03-2021 (measurements, charts, and images), the histological examination report from 11-06-2021, the blood tests from 16-06-2021, and the response from 21-06-2021 regarding the detection of gene mutation. Specifically, she claims that the fetal growth curve, which is created only in combination with multiple tests, was never provided to her.

3. The requested examinations are necessary for investigating any medical negligence of the respondent physician, given that today it has become...

7

to exclude through specialized examinations any genetic, hematological, or hereditary cause of death of the fetus.

4. It is considered that the reported physician did not act out of concern for her as a patient, but

knowingly and intentionally did not provide her with the complete medical file, either to conceal the information that burdens her or to hide that she did not maintain a complete medical file, and in order to prepare for a potential lawsuit against her for medical negligence. Moreover, after her last visit to the physician's office on 22-07-2021, the reported physician never contacted her again, except for one occasion, neither regarding the medication she was following after the physician's prescription, nor concerning the course of the cesarean section, nor out of concern for her psychological state, nor to inquire about the results of her examination by a specialized hematologist to whom she had been referred by the reported physician.

5. Finally, from the history table of the electronic file of the complainant, which the latter submits, it appears that the reported physician accessed the system 4 times from the moment she announced that their collaboration was ending until the day the incomplete file was handed over (period from 13-10-2021 to 21-10-2021), 9 times during the period from 22-10-2021 to 25-01-2022, and one more time on 31-05-2022, at which point she learned of the complainant's pregnancy not from any of her patients, who allegedly recognized the complainant in the waiting room of an unnamed clinic. Following the above, the Authority summoned with its letters numbered Γ/ΕΞ/2669/03-10-2024 and Γ/ΕΞ/2670/03-10-2024, A and B respectively, to a hearing in order to present their views on the case, at the Authority's Department on 09 October 2024, on which date the case was postponed to 30-10-2024. During the session on 30-10-2024 of the Authority's Department, A and her lawyer Stefanos Topalis attended via videoconference, as well as B's lawyer Panagiotis Kotsalis. During the hearing, the parties presented their views and subsequently, they were given a deadline and submitted timely, the complainant the document numbered Γ/ΕΙΣ/9057/22-11-2024, and the reported physician the document numbered Γ/ΕΙΣ/8942/19-11-2024.

Specifically, the complainant with her document numbered Γ/ΕΙΣ/9057/22-11-2024 reiterates what she invoked in her complaint under consideration, as well as her relevant clarifying statement in response to the Authority's document numbered Γ/ΕΞ/2721/27-10-2022, and adds that she never received, despite requesting a copy of her complete medical file, the results of the hematological and hormonal tests conducted during her pregnancy, the medication that was followed, the details of the ultrasounds (measurements and photographs) that were performed at the reported physician's office during the 1st and 2nd trimesters of pregnancy, and the cardiotocogram that was performed two days before the infant's death.

The reported physician, with her document numbered Γ/ΕΙΣ/8942/19-11-2024, reiterates that she provided the complainant with all the results of the regular examinations, namely ultrasounds, cardiotocograms, fetal growth curves, pregnancy reports, at each visit to the office (10 in total), and subsequently referred her to a specialized hematologist for the investigation of the causes of the infant's loss, and that at this moment she holds no documents because she provided everything she had available to the complainant. She also adds that in the complaint under consideration, only the results of the ultrasounds are mentioned as not having been provided and not the entirety of the other examinations, and in any case, at this moment, the reported physician holds no documents because she handed over the relevant file to the complainant. Furthermore, in her memorandum, the reported physician reiterates the claims contained in her document dated 27-09-2022 before the Authority. The Authority, after examining all the elements of the file and the discussions that took place in the session of 30-10-2024, having heard the rapporteur and the clarifications from the assistant rapporteur, who attended without voting rights after thorough discussion.

9

CONSIDERED IN ACCORDANCE WITH THE LAW

1. Whereas, according to the provisions of Articles 51 and 55 of the General Data Protection Regulation (EU) 2016/679 (hereinafter, GDPR) and Article 9 of Law 4624/2019 (Government Gazette A' 137), the Authority has the competence to supervise the implementation of the provisions of the GDPR, this law, and other regulations concerning the protection of individuals from the processing of personal data. Specifically, from the provisions of Articles 57 paragraph 1 point (f) of the GDPR and 13 paragraph 1 point (g) of Law 4624/2019, it follows that the Authority has the competence to address the aforementioned complaint, as the main complaints of the complainant regarding the incomplete

satisfaction of the right of access, unlawful access to electronic health systems, as well as illegal disclosure through the transmission of data to third parties are based on the processing of personal data of the complainant that occurs through automated means, establishing the Authority's competence (Articles 2 paragraph 1 GDPR and 2 Law 4624/2019).

2. Whereas specifically, according to Article 4 point (1) of the GDPR, "personal data means any information relating to an identified or identifiable natural person ('data subject'); an identifiable natural person is one who can be identified, directly or indirectly, in particular by reference to an identifier such as a name, an identification number, location data, an online identifier, or to one or more factors specific to the physical, physiological, genetic, mental, economic, cultural, or social identity of that natural person." Additionally, in Article 4 point (15) of the GDPR, "data concerning health" is defined as personal data related to the physical or mental health of a natural person, including the provision of health care services, and which reveal information about the health status of that person.

3. Whereas further, according to Article 4 point (2) of the GDPR, the processing of personal data is "any operation or set of operations which is performed on personal data or on sets of personal data, whether or not by automated means, such as collection, recording, organization, structuring, storage, adaptation or alteration, retrieval, consultation, use, disclosure by transmission, dissemination or otherwise making available, alignment or combination, restriction, erasure or destruction."

4. Whereas according to Article 4 point (7) of the GDPR, the data controller is "the natural or legal person, public authority, agency, or other body which, alone or jointly with others, determines the purposes and means of the processing of personal data; where the purposes and means of such processing are determined by Union law or the law of a Member State, the controller or the specific criteria for its designation may be provided for by Union law or the law of a Member State."

5. Whereas according to Article 4 point (10) of the GDPR, a third party is "any natural or legal person, public authority, agency, or body other than the data subject, the controller, the processor, and the persons who, under the direct authority of the controller or the processor, are authorized to process personal data."

6. Whereas according to Article 5 paragraph 1 point (a) of the GDPR, "personal data shall be processed lawfully and fairly in a transparent manner in relation to the data subject" (principle of lawfulness, fairness, and transparency).

7. Whereas specifically, in order for a processing operation to be considered lawful (and in accordance with Article 5 paragraph 1 GDPR), at least one of the legal grounds for processing provided in Article 6 paragraph 1 (points (a) to (f)) of the GDPR must apply, namely "1. Processing shall be lawful only if and to the extent that at least one of the following applies: (a) the data subject has given consent to the processing of his or her personal data for one or more specific purposes; (b) processing is necessary for the performance of a contract to which the data subject is party or in order to take steps at the request of the data subject prior to entering into a contract; (c) processing is necessary for compliance with...

11

legal obligation of the data controller, d) the processing is necessary for the protection of vital interests of the data subject or another natural person, e) the processing is necessary for the performance of a task carried out in the public interest or in the exercise of official authority vested in the data controller, f) the processing is necessary for the purposes of the legitimate interests pursued by the data controller or a third party, except where such interests are overridden by the interests or fundamental rights and freedoms of the data subject which require the protection of personal data, especially if the data subject is a child. The provision f) of the first paragraph does not apply to processing carried out by public authorities in the performance of their tasks.

8. Since for personal data to be lawfully processed, that is, processed in accordance with the requirements of the GDPR, the conditions for the application and adherence to the principles of Article 5(1) of the GDPR must be cumulatively met, as also indicated by the ruling of the Court of Justice of

the European Union (CJEU) on 16-01-2019 in case C496/2017 Deutsche Post AG v. Hauptzollamt Köln. The existence of a lawful basis (Article 6 GDPR) does not exempt the data controller from the obligation to adhere to the principles (Article 5(1) GDPR) regarding the legitimacy, necessity, and proportionality as well as the principle of data minimization. In the event that any of the principles set out in Article 5(1) GDPR are violated, the processing in question is deemed unlawful (contrary to the provisions of the GDPR) and the examination of the conditions for the application of the legal bases of Article 6 GDPR becomes unnecessary. Thus, the unlawful collection and processing of personal data does not remedy the existence of a lawful purpose and legal basis.

9. Furthermore, according to Article 9(1) and (2)(h) "1. The processing of personal data revealing racial or ethnic origin, political opinions, religious or philosophical beliefs, or trade union membership, as well as the processing of genetic data, biometric data for the purpose of uniquely identifying a natural person, data concerning health, or data concerning a natural person's sex life or sexual orientation is prohibited. 2. Paragraph 1 does not apply to the following cases: (...) h) the processing is necessary for the purposes of preventive or occupational medicine, assessment of the worker's capacity to work, medical diagnosis, provision of health or social care or treatment, or management of health and social systems and services based on Union law or the law of a Member State or under a contract with a health professional and subject to the conditions and safeguards referred to in paragraph 3. 3. Personal data referred to in paragraph 1 may be processed for the purposes provided for in paragraph 2(h) when such data are processed by or under the responsibility of a professional bound by the obligation of professional secrecy under Union law or the law of a Member State or under rules established by competent national authorities or by another person who is also subject to the obligation of secrecy under Union law or the law of a Member State or under rules established by competent national authorities."

13

10. Furthermore, according to Article 15 of the GDPR, "1. The data subject has the right to obtain from the controller confirmation as to whether or not personal data concerning them is being processed and, if that is the case, the right of access to the personal data and to the following information: a) the purposes of the processing, b) the relevant categories of personal data, c) the recipients or categories of recipients to whom the personal data have been or will be disclosed, in particular recipients in third countries or international organizations, d) where possible, the period for which the personal data will be stored or, if that is not possible, the criteria used to determine that period, e) the existence of the right to request from the controller rectification or erasure of personal data or restriction of processing of personal data concerning the data subject or the right to object to such processing, f) the right to lodge a complaint with a supervisory authority, g) when the personal data have not been obtained from the data subject, any available information as to their source, h) the existence of automated decision-making, including profiling, as referred to in Article 22 paragraphs 1 and 4 and, at least in those cases, significant information about the logic involved, as well as the significance and the envisaged consequences of such processing for the data subject. (...) 3. The controller shall provide a copy of the personal data undergoing processing (...) If the data subject submits the request by electronic means and unless the data subject requests otherwise, the information shall be provided in a commonly used electronic format (...)".

11. Furthermore, in the recent Guidelines 1/2022 of the European Data Protection Board (hereinafter "GDB 1/2022") regarding the rights of data subjects – right of access, it is explained that the right of access consists of three components: a) confirmation as to whether or not personal data is being processed, b) access to that data, and c) provision of information regarding the processing. It is further clarified that the critical time for assessing the existence of the first aforementioned component (i.e., the processing of personal data) by the controller is the time of exercising the right, and its satisfaction requires the existence of personal data (availability) at that time.

12. Furthermore, according to Article 14 of the Code of Medical Ethics (Law 3418/2005) "1. The physician is obliged to maintain a medical record, in electronic or non-electronic form, which contains data that are inextricably or causally linked to the illness or health of their patients. The provisions of

Law 2472/1997 (Government Gazette 50 A') apply to the maintenance of this record and the processing of its data. 2. The medical records must contain the full name, patronymic, gender, age, profession, address of the patient, dates of visits, as well as any other essential information related to the provision of care to the patient, such as, depending on the specialty, their health complaints and the reason for the visit, the primary and secondary diagnosis, or the treatment followed. 3. Clinics and hospitals maintain in their medical records the results of all clinical and paraclinical examinations. 4. The obligation to maintain medical records applies: a) In private practices and other primary health care units in the private sector, for a decade from the last visit of the patient. b) In any other case, for twenty years from the last visit of the patient."

4 See Guidelines 1/2022 of the EDPB regarding the rights of data subjects – Right of access (March 28, 2023), para. 3, available at https://www.edpb.europa.eu/our-work-tools/our-documents/guidelines/guidelines-012022-data-subject-rights-right-access_en (hereinafter "GDB 1/2022").

5 GDB 1/2022, para. 37 et seq. (with reference to the judgment of the Court of Justice of the European Union in case C-553/07, May 7, 2009, *College van burgemeester en wethouders van Rotterdam v. M.E.E. Rijkeboer* regarding the right of access to information regarding recipients or categories of recipients concerning the past): The assessment by the controller of the personal data being processed reflects as closely as possible the situation when the controller receives the request, and the response should cover all data available at that specific time. Therefore, controllers are not obliged to provide personal data that was processed in the past but is no longer available to them.

15

13. Since in this case, regarding the failure to satisfy the right of access, which consists of the non-provision of a complete medical file to the patient, it emerged that the respondent, despite its initial claim in the email dated 13-10-2021 that it does not maintain a record, did maintain a record, which, however, was incomplete (for example, the images and results of the ultrasounds were not kept) and was provided to the complainant in parts, either during her visits to the office of the respondent, or during her referral to a specialist hematologist, or after the request made by her husband on 13-10-2021, at which point the respondent provided the remaining examinations that it had in its records around the end of October to early November 2021, as stated by the respondent itself in the document with reference number Γ/ΕΙΣ/10473/27-09-2022. The complainant, in the document with reference number Γ/ΕΙΣ/12057/24-11-2022 providing clarifications to the Authority, states that during her visits to the office of the respondent for monitoring her pregnancy, she had been given copies of some measurements, as well as some photographs of the ultrasounds, while some of these documents do not bear her name, and in some others, her name has been added manually. She even submits the documents from the medical file that were handed over to her by the respondent, specifically submitting the ultrasound from 02-02-2021 (measurements, diagrams, and images), the ultrasound from 16-03-2021 (measurements, diagrams, and images), the histological examination report from 11-06-2021, the blood tests from 16-06-2021, and the response from 21-06-2021 regarding the detection of a gene mutation.

14. Furthermore, from the study of all the elements of the case and based on the considerations mentioned above, the Authority finds, in this case, that a violation of the complainant's right of access cannot be established, according to the provisions of Article 15 of the GDPR. This is because at the time of exercising the right of access, it emerged that the medical documents maintained in the respondent's records (in the medical file of the complainant) on the day the access request was made (13-10-2021) were timely provided to the complainant by the respondent physician. Regarding the other medical documents, those that had already been delivered to the complainant during her visits to the respondent's office as originals had not initially been included in the respondent's records, while those provided to the complainant during her referral to a specialist hematologist were no longer maintained in the respondent's records on the day the access request was made. For this reason, the complainant's assertion that the respondent's obligation cannot be negated by the fact that the complainant already had certain documents in her possession is rejected as irrelevant, since in any case, the respondent was obliged to provide her with a copy of all medical acts that had been

performed in her office, given that a critical element for the satisfaction of the right of access, as mentioned above, is precisely the existence of personal data in the records of the data controller at the time of exercising the right. Finally, the documents that were not provided after the complainant's access request on 13-10-2021 either had not initially been included in an automated processing/filing system of the respondent (Article 14 of Law 3418/2005 - Code of Medical Ethics), so that the latter could satisfy the relevant request for the provision of copies, or had been included and subsequently provided to the complainant. In light of the above, the complainant's assertion that the requested examinations are necessary for investigating any medical negligence of the respondent physician is also rejected as unfounded, given that a critical element for the satisfaction of the right of access is the existence of personal data at the time of exercising the right, and not the purpose of using them. Therefore, according to the above detailed considerations regarding this aspect, the complaint is deemed to be rejected.

15. Furthermore, regarding the unlawful access to the complainant's individual electronic file via the website <https://ehealth.gov.gr>, the respondent admits that it indeed had access to it during the relevant time periods. Specifically, it claims that the access it obtained during the months of October-November 2021 was due to its attempt to...

17

She gathered the material in view of delivering the file to her husband, in order to be able to provide "accurate information regarding the factual circumstances and the therapeutic protocol that was followed," but also to provide "all information if requested by the next attending physician." Furthermore, for the upcoming period, during the months of December 2021-May 2022, the respondent invokes reasons of medical (scientific) as well as human interest, as she was tormented by the loss of the fetus, and as soon as she learned from a mutual acquaintance that the complainant was pregnant again, she accessed her electronic file in order to "remember exactly what had happened in view of the potential new pregnancy," which was now being monitored by another doctor. However, the aforementioned claims of the respondent cannot justify her access to the complainant's personal electronic file, given that due to the termination of their medical services contract that had preceded, there was no authorization for this access, and the access was unlawful and occurred without the complainant's knowledge.

16. Since the AMKA, according to the case law of the Authority, constitutes a simple personal data, as the processing of the AMKA can immediately or indirectly identify the identity of the subject. The AMKA does not, by itself, reveal the slightest sensitive personal data of the respective interested subject. However, through the use of the AMKA, it can clearly reveal the age of the respective interested party, which also constitutes a simple personal data. In this case, the respondent doctor gained access to the electronic health file of the complainant and processed the personal data concerning her health status, without it being established that the consent of the subject had been obtained. Specifically, it is found that the respondent, as the data controller, specifically registered the identification details of the complainant, which she kept in her professional file (indicatively name, surname, AMKA, etc.) and which she does not claim to have deleted, and subsequently searched for information through these in the electronic health system. In the aforementioned ways, the respondent processed special category data (health data) of the complainant, without there being a legal basis for this, without the complainant's knowledge, without providing her with medical services anymore, and without any of the purposes of Article 9 paragraph 2 being applicable, especially since the complainant had already informed the respondent that she did not wish to continue the contract for the provision of health services (see also Article 9 paragraph 3 GDPR). For the above reasons, the actions of processing by the respondent took place in violation of Article 5 paragraph 1 subparagraph a of the GDPR.

17. Finally, from the evidence in the file, it was not proven that there was indeed a transfer to third parties of health data by the respondent, and therefore the relevant claim of the complainant is rejected as evidently unfounded.

18. Consequently, it is established that the respondent, as the data controller a) did not have at her disposal all the requested personal data from the complainant's medical file, except for the medical

documents she delivered to her husband following his request on 13-10-2021, as well as the details she used to access the electronic health system (name, surname, AMKA, etc.), at the time the right of access was exercised, b) engaged in unlawful and non-legal processing of the complainant's data (AMKA, full name, etc.), which she kept in her professional file, by entering them into the electronic health system and searching for information.

19

information regarding the health status of the complainant, thereby obtaining unfair, unethical, and illegal access to the medical information of the complainant, without her knowledge and without being authorized to do so, in violation of the principles of legality and objectivity of Article 5(1)(a) of the GDPR, and c) finally, there is no evidence of the transmission of this data to third parties. Following the above, the examination of the conditions for the application of the legal bases of Article 6 of the GDPR, as previously mentioned in Recital 8 of this document, is unnecessary.

19. Since the violation of the fundamental principles for processing, as detailed above, incurs the imposition of administrative sanctions under Article 83(5)(a) of the GDPR. According to the GDPR (Recital 148), in order to strengthen the enforcement of the rules of this Regulation, sanctions, including administrative fines, should be imposed for each violation of this Regulation, in addition to or instead of the appropriate measures imposed by the supervisory authority in accordance with this Regulation.

20. Since the Authority, based on the above, considers that the imposition of a corrective measure is insufficient to restore compliance with the provisions of the GDPR that have been violated and that, based on the circumstances established, an additional effective, proportionate, and deterrent administrative monetary fine must be imposed under Article 83 of the GDPR, both to restore compliance and to sanction the illegal behavior.

21. Since furthermore the Authority took into account the criteria for assessing the fine as defined in Article 83(2) of the GDPR, paragraph 5 of the same article applicable to the present case, and the Guidelines 4/2022 of the European Data Protection Board for the calculation of administrative fines for the purposes of Regulation 2016/679, which were approved on 24/5/2023, as well as the factual data of the examined case and in particular:

10 See EDPB 29, Guidelines on the application and setting of administrative fines for the purposes of Regulation 2016/679 WP253, p. 6

20

i) The fact that the complainant, in her capacity as data controller, violated the principle of legality and objectivity as provided in Article 5(1)(a), that is, she violated a fundamental principle of the GDPR for the protection of personal data.

ii) The fact that adherence to the principles set forth in Article 5(1) of the GDPR is of paramount importance, primarily the principle of legality, so that if this is absent, the processing becomes unlawful from the outset, even if the other processing principles have been adhered to.

iii) The fact that the complainant, according to her own admissions, intentionally engaged in unlawful processing of the personal data of the complainant in order to gain knowledge of information concerning her health status, that is, the violation was caused knowingly by the complainant physician and cannot be attributed to her negligence.

iv) The fact that the unlawful processing did not occur once, but repeatedly during the period under review (October 2021 - May 2022).

v) The fact that there are no previous related violations by the complainant.

vi) The fact that the complainant did not delay in responding to the Authority's documents and cooperated in providing clarifications.

vii) The fact that the unlawful processing resulted in the complainant gaining access to data concerning

the health status of the complainant, that is, to data of special categories, for which a stricter protection framework applies.

viii) The fact that from the evidence presented to the Authority, based on which it established the aforementioned violation of the GDPR, it does not appear that the data controller caused material damage to the affected individual through the contested processing.

22. Based on the above, the Authority unanimously decides that an administrative sanction, as mentioned in the operative part, must be imposed on the complainant as the data controller, which is deemed proportionate to the severity of the violation.

21

FOR THESE REASONS

The Authority,

Imposes on the complainant B as the data controller the effective, proportionate, and deterrent administrative monetary fine that is appropriate in this specific case, according to its specific circumstances, in the total amount of five thousand (5,000) euros, for the above-established violation of Article 5(1)(a), as specified above, in accordance with Articles 58(2)(h) and 83(5)(a) of the GDPR.

The Deputy President

The Secretary

Georgios Batzalexis

Irini Papageorgopoulou